

CITY GENERAL HOSPITAL & CLINIC

Department of Otorhinolaryngology & Pulmonology

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PRESCRIPTION SLIP

Patient Details		Clinical Metadata	
Patient Name:	Aryan Verma	Date:	August 28, 2026
Age / Gender:	17 Yrs / Male	Weight / BP:	68 kg / 118/78 mmHg
Pulse:	84 bpm	SpO ₂ (Room Air):	98%

Chief Complaints & Clinical Presentation

- Bilateral nasal congestion and obstruction × 4 days
- Sore throat, painful deglutition (difficulty swallowing), and enlarged tonsils × 3 days
- Mild respiratory discomfort and perceived breathlessness secondary to upper airway obstruction and post-nasal drip

Clinical Diagnosis (Provisional)

- Acute Follicular Tonsillitis with Acute Rhinosinusitis (Mild upper-airway reactive breathlessness)

Rx (Medications)

- Tab. Amoxicillin + Clavulanic Acid (625 mg)**
 - Dosage:** 1 tablet twice daily (BD) after food × 5 days
 - Instructions:** Complete the full antibiotic course without interruption.
- Tab. Paracetamol (650 mg)**
 - Dosage:** 1 tablet up to three times daily (TDS) as needed for throat pain or fever.
- Tab. Levocetirizine (5 mg) + Montelukast (10 mg)**
 - Dosage:** 1 tablet once daily (OD) at bedtime × 7 days
 - Instructions:** For relief from nasal mucosa swelling and airway inflammation.
- Nasal Spray Xylometazoline (0.1%)**
 - Dosage:** 1–2 sprays in each nostril twice daily (BD) × 3 to 4 days max
 - Instructions:** Do not exceed 4 consecutive days to avoid rebound congestion (*Rhinitis medicamentosa*).
- Inhaler / Respules Levosalbutamol + Ipratropium Bromide (SOS) (If indicated)**
 - Dosage:** 2 puffs via spacer as needed (SOS) if acute breathlessness or wheezing occurs.

General & Supportive Advice

- Warm saline gargles or povidone-iodine gargles 3–4 times daily.
- Steam inhalation with plain water twice daily.
- Stay well hydrated; consume lukewarm fluids, clear soups, and avoid cold/chilled beverages or oily foods.

Red-Flag Warnings & Emergency Instructions

Seek immediate emergency medical evaluation if you experience:

- Rapid worsening of breathlessness, stridor, or inability to speak full sentences
- Severe difficulty swallowing saliva (drooling) or inability to open mouth (*Trismus*)
- Persistent high-grade fever ($> 102^{\circ}\text{F}$) unresponsive to antipyretics
- Drop in oxygen saturation ($\text{SpO}_2 < 94\%$)

Follow-up: Review in OPD after 5 days or immediately if red flags appear.

Doctor's Signature & Seal

Dr. R. K. Sharma, MD, DLO (ENT)

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